NORTHGATE VALLEY MEDICAL CENTER

Department of Diagnostic Radiology
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RADIOLOGY REPORT

Report Status: FINAL

PATIENT NAME	MRN	DOB	AGE / SEX
OKAFOR, LINDA M.	004827193	08/22/1967	58 Y / F
ACCESSION #	**PATIENT CLASS**	**LOCATION**	**PRIORITY**
RAD26-0203-3381	Emergency	ED - Bay 14	STAT
REFERRING PHYSICIAN	**ORDERED**	**EXAM PERFORMED**	**REPORT SIGNED**
Priya Raman, MD (Emergency Medicine)	02/03/2026 08:12	02/03/2026 08:41	02/03/2026 09:17

EXAM: XR HUMERUS LEFT, 2 VIEWS

CPT 73060

CLINICAL HISTORY

58-year-old female who slipped on ice and fell onto the left arm this morning. Pain, swelling and deformity of the mid upper arm. Unable to lift the arm. Evaluate for fracture.

COMPARISON

None available.

TECHNIQUE

Frontal (AP) and lateral radiographs of the left humerus were obtained. The shoulder and elbow joints are included in the field of view.

FINDINGS

Bones: There is an acute, transverse fracture of the mid-diaphysis (mid-shaft) of the left humerus, located approximately 14.5 cm distal to the greater tuberosity. The distal fracture fragment is displaced laterally by approximately 6 mm (about one-third of the shaft width) with approximately 18 degrees of apex-lateral (varus) angulation and 4 mm of shortening. No comminution or butterfly fragment is identified. There is no extension of the fracture to the proximal or distal articular surfaces.

Shoulder: The humeral head is normally located within the glenoid. No fracture of the humeral head, anatomic neck, surgical neck, or greater/lesser tuberosity. The acromioclavicular joint is intact. The visualized clavicle and scapula are intact.

Elbow: No fracture of the distal humerus, radial head or olecranon. Radiocapitellar and ulnohumeral alignment is maintained. No joint effusion; no elevated anterior or posterior fat pad.

Bone mineralization: Mild diffuse osteopenia.

Soft tissues: Soft tissue swelling surrounds the fracture site. No subcutaneous gas or radiopaque foreign body.

IMPRESSION

1. Acute, transverse, mildly displaced fracture of the left humeral mid-shaft with approximately 18 degrees of varus angulation, 6 mm lateral displacement and 4 mm shortening (AO/OTA 12-A3).
2. No dislocation of the left shoulder or elbow. No intra-articular extension.
3. Mild osteopenia.
4. Humeral shaft fractures are associated with radial nerve injury; correlation with the neurovascular examination of the left upper extremity is recommended.

Findings were discussed with Dr. Priya Raman in the Emergency Department by telephone at 09:10 on 02/03/2026.

Interpreted and electronically signed by:
Elena Vasquez, MD
Attending Radiologist, Musculoskeletal Imaging

Signed: 02/03/2026 09:17
Workstation: NVMC-RAD-07

SYNTHETIC TEST DOCUMENT - fictitious patient, clinicians and facility. Created for software testing only. Not a medical record.